



OFFICIAL

# Bladder Management – Continence Procedure

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## 1. Scope

| Site                   | Operational Area | Applicable to             |
|------------------------|------------------|---------------------------|
| Armada Kalamunda Group | All services     | Nursing and Allied Health |

## 2. Purpose

To establish minimum practice standards for the assessment and management of bladder function including:

- Continence assessment.
- Bladder assessment.
- Urinary retention.
- Continence management.
- Bladder training exercise.
- Pelvic floor muscle exercises.

### 2.1 Information excluded from this procedure includes:

- Urinary catheterisation.
- Intermittent and self-catheterisation.
- Urinary catheter patient self-care.

## 3. Procedure

### 3.1 Assessment

#### 3.1.1 Normal bladder function

- Voiding at three-five hourly intervals.
- Nocturia one-two times / night.
- No urgency.
- No dysuria (discomfort, burning on urination).
- No post void residual.
- No incontinence.

## 3.2 Types of Incontinence

Urinary incontinence may be classified into five main types.

### 3.2.1 Stress Incontinence

- Urine loss relating to an increase in abdominal pressure exerted on the bladder in the absence of a bladder contraction; may occur during physical exertion such as coughing, running or jumping.
- Possible causes include:
  - Childbirth.
  - Chronic constipation.
  - Post urology surgery.
  - Obesity.
  - Chronic cough.

### 3.2.2 Urge Incontinence

- Uninhibited bladder contractions which may result in urgency with or without incontinence. Also known as Overactive bladder, Unstable bladder, Detrusor instability.
- Symptoms may include:
  - Urgency.
  - Frequency.
  - Nocturia.
- Possible causes include:
  - Acute urinary tract infection (UTI).
  - Renal or bladder stones.
  - Bladder tumours or lesions.
  - Constipation or faecal impaction.
  - Benign prostate hyperplasia (BPH).
  - Idiopathic.
  - Neurogenic: i.e. spinal cord lesion, cerebro-vascular accident, multiple sclerosis or Parkinson's Disease.

### 3.2.3 Overflow Incontinence

- Dribbling and / or continuous leakage associated with incomplete bladder emptying due to either underactive bladder or bladder outlet obstruction.

Symptoms may include:

- Frequency.
- Nocturia.
- Hesitancy.
- Weak urinary stream (dribbling).

Possible causes include:

- Underactive bladder due to central nervous system damage.
  - Diabetic neuropathy, Multiple sclerosis or spinal cord injury.

- Bladder outlet obstruction.
  - Urethral stricture, Benign Prostate Hyperplasia or constipation.
- Decreased bladder contractility due to medications
  - Anaesthetic agents, Antidepressants or ACE inhibitors.

### 3.2.4 Functional incontinence

- Urinary incontinence in the absence of a pathological cause.
- Possible causes may include:
  - Impaired – mobility, dexterity or cognition.
  - Environmental issues – lighting.
  - Toilet facilities – signage, access or location.

### 3.2.5 Transient Incontinence

- New onset or acute incontinence as a result of reversible or transient factors. Transient incontinence may result in persistent incontinence.
- Elderly patients may be most at risk, although patients of ages may be affected.
- Possible causes as identified by the acronym DIAPPERS, may include:

Table 1 Possible causes of transient incontinence

| Acronym                 | Possible Cause(s)                                                               |
|-------------------------|---------------------------------------------------------------------------------|
| Delirium                | Resulting from medications, surgery or acute illness.                           |
| Infection               | Symptomatic urinary tract infection.                                            |
| Atrophic vaginitis      | Post-menopausal urogenital atrophy.                                             |
| Pharmacology            | Medications affecting bladder functioning.                                      |
| Psychological disorders | Depression or sleep deprivation.                                                |
| Excessive urine output  | Large fluid intake, endocrine disorders or medications e.g. diuretics.          |
| Restricted mobility     | Arthritic pain, fear of falling, post prandial hypotension, poor mobility aids. |
| Stool impaction         | Causing both urinary and faecal incontinence.                                   |

- Transient incontinence may ultimately result in chronic persistent incontinence.
- Most often occurs in the elderly patient but patients of all ages may be affected.

### 3.3 Continence assessment

Determining the patient's continence status requires collaboration with the multidisciplinary team to determine the characteristics of presenting continence issues and develop appropriate management strategies.

### 3.4 Patient Admission

- Determine patient's current continence status on admission.
- Identify, review and implement current continence management program if appropriate.
- Determine if patient is acutely ill.
  - Reassess as condition improves.
- If No to any of these details proceed to **Four Key Questions**.

### 3.5 Four Key Questions

Ask all patients the following questions:

1. Do you ever have accidents with your bladder or leak urine?
2. Do you find you have to rush to the toilet or are unable to get there in time?
3. Has it been longer than four-six hours since you last passed urine?
4. Do you have problems with constipation, diarrhoea or have bowel accidents?

If Yes to any question proceed to **Initial Assessment Plan**.

If yes to question four, refer to guidelines in relation to Bowel Management.

### 3.6 Initial Assessment Plan

Undertake assessment for appropriate management and / or referral to specialised services:

### 3.7 Perform Urinalysis

- Request patient to void.
  - Measure contents.
- Undertake a dipstick urinalysis and proceed to mid-stream urine testing as indicated by results.
- If patient presents with urinary tract infection, reassess when treatment is complete.

### 3.8 Post-void Residual Volume

- Assess abdomen for signs of bladder distention.
- Assess perineal skin integrity.
- Check post void residual.
  - Refer to [Appendix 2: Bladder Assessment and Scanning](#).
- If the patient presents with total bladder volume exceeding 500mL prioritise bladder emptying.

### 3.9 Day Bladder Diary

- Undertake to determine Incontinence Type and commence Appropriate Interventions.
- Analyse bladder diary information to identify patterns in voiding and incontinence in relation to:
  - Volume and frequency
  - Patient awareness of needing to void and fluid balance status
  - Document assessment in the patient health record.

Refer to appendix 1 Incontinence Type by Bladder Assessment Pattern.

### 3.10 Specialised Continence Assessment

Following Initial Continence Assessment on patient admission, a referral may be required for specialised assessment and treatment interventions.

### 3.11 Physiotherapy Continence Assessment

The following information may also be considered:

Table 2 Physiotherapy Continence Assessment

| Subjective History                                                                                                                                  | Symptoms                                                                                                                                                                                                                    | Assessment                                                                                                                                                                                                                                                                                                                                    |
|-----------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Presenting complaint<br>Medical history<br>Medications / Interactions<br>Surgical history<br>Obstetric history<br>Hormonal status<br>Investigations | <ul style="list-style-type: none"> <li>• Urinary symptoms</li> <li>• Voiding function / dysfunction</li> <li>• Pelvic organ prolapse</li> <li>• Ano-rectal symptoms</li> <li>• Defecation function / dysfunction</li> </ul> | <ul style="list-style-type: none"> <li>• Demographic information</li> <li>• Exercise behaviours</li> <li>• Validated evaluation tools:                             <ul style="list-style-type: none"> <li>• Quality of life</li> <li>• Symptom severity</li> <li>• Functional activity</li> <li>• Psychosocial factors</li> </ul> </li> </ul> |

### 3.12 Physical Examination

- Visual observation of the perineum and introitus.
- Digital palpation of the pelvic floor muscles and connective tissue.
- Assessment of muscle quality, strength, function and pain.
- Ano-rectal assessment of the anal sphincter and Pubo Rectalis muscle.
- Real time ultrasound imaging of the bladder and pelvic floor muscles.
- Other biofeedback modalities e.g. surface emg; pressure biofeedback.
- Uroflowmetry - if any voiding dysfunction reported.
- Documentation.
- Develop a management plan with short- and long-term goals for treatment based on:
  - The patient's and the physiotherapist's goals.
  - Analysis of continence assessment.

### 3.13 Patient Monitoring

Individualised management plan to be documented in the patients' health records as soon as is practicable. At a minimum, the plan must consider:

- Patient history and diagnosis for clinical conditions, medications, psychosocial and cultural factors that could influence observations.
- Presence of comorbidities and treatment.
- Frequency and specific observations.
- Site requirements, patient education and consent.

Refer to [Appendix 3: Appropriate Interventions by Incontinence Type](#)

## 4. Bladder Assessment and Scanning

- Explain procedure and obtain appropriate consent.
- Perform hand hygiene at all appropriate moments throughout procedure.
- Position patient in supine for bladder assessment and scanning procedures.
- Maintain privacy and dignity throughout procedure

Refer to the [WA Health infection prevention and control policies and guidelines](#) and [Guidelines for Reprocessing Ultrasound Transducers](#) for all cross infection considerations relevant to ultrasound transducers.

Ensure to decontaminate bladder scan equipment as per manufacturer instructions.

Undertake hydration status and bladder volume assessment for patients who have not voided within four hours of admission, surgery or removal of indwelling catheter.

### 4.1 Bladder Assessment

Bladder assessment through abdominal examination is undertaken to assess and determine the:

- Extent of any suspected bladder distention.
- Presence of any surgical scars or palpable masses.

#### 4.1.1 Bladder assessment Procedure

- Examine the patient's abdomen sequentially for contour, distension, scars, engorged veins, visible peristalsis and masses.
- Auscultate all four quadrants for active bowel sounds.
- Percuss all four quadrants systematically to evaluate abnormal sound.
- If there is dullness over the symphysis pubis, continue percussing upward and outward to estimate bladder borders.
- Palpate all four quadrants superficially, then deeper to identify rebound tenderness or abnormality.
- To identify a distended bladder, begin at the symphysis pubis and move upwards and outwards to estimate bladder borders.
- The distended bladder is palpated as a smooth, round firm mass, extending as far as the umbilicus.

### 4.2 Bladder Scanning

- Bladder scanning is undertaken to determine the volume of urine contained within the bladder either relating to urinary retention or for post void residual (PVR) volume.
- Scanning using a specifically designed ultrasound device is a non-invasive alternative to inserting an intermittent catheter for measuring PVR volume.
- Bladder scanning is not suitable for pregnant women; and patients with the following:
  - Suprapubic region: wounds, sutures/staples, scar tissue and lesions.
  - Indwelling catheters.

- Abdominal ascites.
- Large abdominal wounds.

Patients on Short Stay Joint Pathway with Bladder volume greater than 500mls, encourage to void using the toilet, as long as the patient is not in any discomfort to leave and reassess in 1 hour. If bladder volume reached 750mls insertion of IMC.

Table 3 Bladder scan guidelines

| Bladder Volume      | Reassess Patient                                                                                                                                  | Other                                                                                                                 |
|---------------------|---------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------|
| Less than 300 mL    | in 2 hours                                                                                                                                        | Review patient's hydration status and continue to assess.<br>Assess sensation to void.                                |
| 300 to 500 mL       | in 1 hour                                                                                                                                         | Arrange urgent review (if bladder volume 400-500mL)<br>Implement Non-invasive Strategies<br>Assess sensation to void. |
| Greater than 500 mL | Liaise with senior nursing staff and the Medical Officer.<br>Consider inserting an indwelling or intermittent catheter as clinically appropriate. |                                                                                                                       |

#### 4.2.1 Bladder Scanning – Equipment Required

- Portable bladder scanner.
- Alcohol wipes.
- Water soluble gel.
- Tissues.

#### 4.2.2 Fluid balance status review

- Calculate hydration status: fluid input (oral + intravenous) **minus** fluid output (urine + wound).
- Consider normal voiding pattern, time of last void and length of time fasting (if applicable).
- Ensure patient is pain free and discuss urge to void.

#### 4.2.3 Bladder scanning Procedure

- Refer to the site specific and / or scanner specific instructions.
- Clean scan head with an alcohol wipe.
- Switch scanner on and select appropriate gender.
  - Select Male option for women who have undergone hysterectomy.
- Palpate the symphysis pubis.
  - Apply generous amount of gel to abdomen.
- Place the scan head midline 3-4 cm above the symphysis pubis.
- Observe scanner for bladder volume.
  - Adjust scan head if inaccurate display visualised.



- On completion of bladder scan assist patient remove gel, redress and reposition.

#### 4.2.4 Decontaminate Scan Transducer

- Switch off bladder scan machine.
  - Wipe the water-soluble gel from scan head with tissues.
- Thoroughly clean scan head with alcohol wipes.
- Ensure to decontaminate bladder scan equipment as per manufacturer instructions.

### 5. Post Procedure for Bladder Assessment and Scanning

- Analyse the assessment and/or scan in relation to the patient's current clinical status.
- Document the assessment and/or scan in the patient's health record.
- Be aware that even if the patient is comfortable and there is no obvious distension, a PVR may still be present due to decreased sensation.
- Continue to monitor the patient according to physical assessment and bladder scan guidelines.

#### 5.1 Urinary Retention

##### 5.1.1 Causes and Pre-disposing Factors

- Patients at risk of urinary retention may include:
  - Post-surgery.
  - Following removal of indwelling catheter.
  - Acute pain service therapy.
  - Spinal epidural anaesthetic.
  - Post urology or gynaecological procedures.

Table 4 Causes of obstruction of lower urinary tract or below the bladder neck

| Intrinsic Causes                                                                                                                                                                                                                                                          | Extrinsic Causes                                                                                                                     |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------|
| <ul style="list-style-type: none"> <li>• Enlarged prostate</li> <li>• Bladder / urethral stone</li> <li>• Haematuria with clot retention</li> <li>• Surgical disturbance of bladder innervation</li> <li>• Urethral stricture / meatal stenosis / paraphimosis</li> </ul> | <ul style="list-style-type: none"> <li>• Organ prolapse</li> <li>• Pelvic mass</li> <li>• Constipation / faecal impaction</li> </ul> |

- **Neurological** disturbances to the pathways between the brain, autonomic nervous system and somatic nerves. Interruption to these pathways can result in urinary retention.
- Examples may include:
  - Spinal cord trauma/ spina bifida / cauda equine syndrome.
  - Parkinson's disease/ multiple sclerosis.
  - Cerebral vascular accident.
  - Diabetic autonomic neuropathy.
  - Radical pelvic surgery / post colorectal surgery.
- **Pharmacological** agent interactions:

- Examples may include:
  - Anticholinergic agents e.g. oxybutynin, atropine or amitriptyline.
  - Antiparkinsonism e.g. levopoda.
  - Antipsychotics e.g. haloperidol.
  - Opiates e.g. morphine.
  - Sympathomimetic agents e.g. pseudoephedrine.
- **Post Void Residual (PVR) volume:** is the amount of urine remaining within the bladder 10-15 minutes after voiding.
- Proper bladder emptying is the result of many factors and may vary from person to person.
  - The patient's combined total voided volume **plus** residual volume are to be considered when assessing the clinical significance of the PVR.
- Consider the following criteria, and in conjunction with the patient's clinical condition with regards to ceasing Trial of Void:
  - Void must be double the volume of the residual urine scanned providing total volume (void plus residual) does not exceed 800mL.
  - Two consecutive residual volumes of Less than 100mL.

## 5.2 Urinary Retention Management

- Management of Urinary Retention will depend on patient assessment outcome.
- Refer to [Appendix 1 Urinary Retention Management Flow Chart](#) and [section on Bladder Assessment and Scanning](#).

## 5.3 Non-Invasive Strategies

- Ensure patient privacy and provide comfort.
- If clinically stable, sit patient on toilet / commode or stand out of bed.
- If appropriate, assist the patient into a warm shower.
- Turn on a tap so the patient can hear the sound of water.

## 5.4 Urethral Catheterisation

- Ongoing management will depend on amount of residual urine.
- If urethral catheterisation is required to relieve bladder distention, monitor post insertion urine output at 15 minutes and one hour.

Table 5 Urethral catheterisation monitoring post insertion

| Bladder Volume       | Timeframe                                             |
|----------------------|-------------------------------------------------------|
| Greater than 500 mL  | Indwelling catheter minimum 48 hours                  |
| Greater than 750 mL  | Indwelling catheter minimum 72 hours                  |
| Greater than 1000 mL | Liaise with the Continence Advisor or Medical Officer |

# 6. Continence Management

On completion of Continence and / or Bladder Assessment an individual Continence Management plan may include the one or more of the following:

| General Considerations                                                                                                                   | Urine Containment Strategies                                                                                                |
|------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------|
| <ul style="list-style-type: none"> <li>• Fluid Intake</li> <li>• Environment</li> <li>• Skin Care</li> <li>• Bowel Management</li> </ul> | <ul style="list-style-type: none"> <li>• Absorbent Products</li> <li>• Uridome</li> <li>• Intermittent Catheters</li> </ul> |

### 6.1 Fluid Intake

- Promote adequate fluid intake to prevent constipation, dehydration or urinary tract infection.
- Encourage fluid intake of approximately two litres / 24 hours unless medically contraindicated.
- Monitor fluid balance chart accurately to objectively verify fluid intake and elimination patterns.
- Educate the patient regarding potential diuretic effect of caffeinated drinks:
  - Encourage reduction in caffeine intake.
  - Drastic reduction in caffeine may cause headache.
  - Continue warm caffeinated drinks if required as part of normal bowel regime.
- Document individual patient requirements in the Nursing Care Plan and in the Integrated Notes.

### 6.2 Environment

Modify environment to facilitate continence:

- Ensure the patient has easy access to toilet, commode or urinal.
- Ensure the patient has access to a call bell.
- Assist the patient when the patient is unable to manage independently.
- Provide privacy and dignity for the patient at all times.
- Encourage easily removed clothing where possible.
- Document individual patient requirements in the Nursing Care Plan and in the Integrated Notes.

### 6.3 Skin Care

Assess the patient's skin at groins, inner thighs, perineal and perianal areas minimum daily.

Cleanse the patient's skin after urinary and / or faecal incontinence.

- Assess the perineal area for signs of:
  - Moisture.
  - Impaired skin integrity.
  - Rash.
  - Dryness of skin.

## 7. Bowel Management

- Intervene if bowels not open for three days.

- Liaise with the RMO and document individual patient requirements in the Nursing Care Plan and in the patients Integrated Notes.
- Ensure bowel management does not inhibit voiding.
  - Constipation or impaction can inhibit voiding by causing retention and high residual volume.

### 7.1 Absorbent Products

Ensure to select the most appropriate product to meet patient, staff and organisational preferences for incontinence management.

Consider the following when selecting an absorbent product:

- Incontinence type and severity.
- Patient functional ability, preferences and comorbidities.
- Previous experience of incontinence management.
- Caregiver availability.

### 7.2 Uridome

- Uridome is a non-invasive urine collection system for incontinent male patients.
- May also be known as penile sheath or external condom drainage system.
- Consider the following when selecting a Uridome:
  - Patient cognitive status, functional ability, preferences and comorbidities.
  - Patient penis size and length suitability, a significantly retracted penis may be unsuitable.
  - Patient genital area skin integrity and existing incontinence management strategies.
- Ensure good genital hygiene is maintained.
- Replace Uridome every 24 hours and may be synchronised with washing and showering routines.

#### 7.2.1 Patient Monitoring

- Uridome use may be associated with lower incidence of urinary tract infection in comparison with indwelling catheter use.
- Uridome users require monitoring for:
  - Skin integrity and penile tissue damage.
  - Uridome kinking and leaking.
  - Urinary tract infection

#### 7.2.2 Uridome Procedure – Equipment Required

- Uridome measuring guide.
- Appropriate size uridome.
- Non-sterile gloves.
- Suitable drainage bag with stand or leg attachment as required.
- Patient hygiene requirements: soap, bowl, flannel, towels etc.
- Hair clippers.

- Skin prep swab.

### 7.2.3 Uridome Procedure

- Explain the procedure and obtain appropriate consent.
- Perform [hand hygiene](#) at all appropriate moments throughout procedure.
- Maintain patient privacy and dignity throughout procedure.
- Position the patient in supine.
- Wash the genital area and trim or guard pubic hair as required.
- If applicable, gently retract the foreskin, wash and dry glans penis.
  - Reposition the foreskin.
- Apply skin prep if required, allow skin to air dry.
- Place the sheath at the tip of the penis, ensuring foreskin is pulled forward over glans penis.
- Slowly roll the sheath along the penis leaving a 12mm gap at the tip.
- Apply firm pressure for 10 seconds around the applied
  - Uridome to activate adhesive.
- Connect the drainage bag and stand or leg attachment as appropriate.

### 7.3 Intermittent Catheters

- An intermittent catheter is inserted into the bladder to remove urine on periodic basis.
- Episodic intermittent catheter insertion will be required as appropriate to prevent bladder over-distention and manage incontinence.
- Refer to the [AKG Urinary Catheter Management Procedure](#) for indwelling, intermittent and self-catheterisation

## 8. Policy context

The following documents are required to give effect to this procedure:

- HDWA
  - [WA Health infection prevention and control policies and guidelines](#)
- EMHS
  - [Patient Identification and Procedure Matching EMHS: 21](#)
  - [Consent Policy EMHS: 5](#)

## 9. Supporting information

The following documents support the implementation of this procedure:

- [Guidelines for Reprocessing Ultrasound Transducers](#)
- [Urinary Catheter Management Procedure](#)
- [Hand Hygiene Policy](#)
- [Decontamination of Diagnostic Ultrasound Transducers](#)

## 10. Compliance, monitoring and evaluation

Compliance against this procedure will be evaluated with the monitoring of clinical incidents relating to bladder management and the level of hospital-acquired infections specific to this topic.

## 11. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 3 – Action 3.14 Reprocessing of reusable devices
- Where reusable equipment, instruments and devices are used, the health service organisation has:
  - a. Processes for reprocessing that are consistent with relevant national and international standards, in conjunction with manufacturers' guidelines
  - b. A traceability process for critical and semi-critical equipment, instruments and devices that is capable of identifying
    - the patient
    - the procedure
    - the reusable equipment, instruments and devices that were used for the procedure
- Standard 6 – Action 6.5 Correct identification and procedure matching
- The health service organisation:
  - a. Defines approved identifiers for patients according to best-practice guidelines
  - b. Requires at least three approved identifiers on registration and admission; when care, medication, therapy and other services are provided; and when clinical handover, transfer or discharge documentation is generated
- Standard 6 – Action 6.7 Clinical handover
- The health service organisation, in collaboration with clinicians, defines the:
  - a. Minimum information content to be communicated at clinical handover, based on best-practice guidelines
  - b. Risks relevant to the service context and the particular needs of patients, carers and families
  - c. Clinicians who are involved in the clinical handover

## 12. Glossary

The following definitions are relevant to this policy.

| Term               | Definition                                                                                                                                                                                                                                                                                                     |
|--------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Urinary continence | The ability to store urine in the bladder and voluntarily excrete as required and where socially appropriate.                                                                                                                                                                                                  |
| Urinary retention  | This is the failure to completely empty the bladder. It is a common occurrence and can lead to bladder distention with significant outcomes for the patient. Distention injuries are avoidable if the patient's bladder is monitored.<br>chronic<br>It may be clinically diagnosed as either Acute or Chronic. |

| Term              | Definition                                                                                                                                                                                                                                                                                                                                                                                                                                             |
|-------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Acute retention   | This is a painful percussible bladder, when the patient is unable to pass any urine” and may present with symptoms of: <ul style="list-style-type: none"> <li>• Voiding small volumes or dribbling incontinence.</li> <li>• urgency or frequency.</li> </ul> Acute urinary retention should be considered an emergency. Failure to empty the bladder can cause excess reflux of urine to the ureters with resulting renal damage and / or dysfunction. |
| Chronic retention | This is “a non-painful bladder which remains palpable or percussable after the patient has passed urine”<br>It requires differentiation from transient voiding difficulties, and may develop to acute urinary retention associated with other factors such as UTI or constipation                                                                                                                                                                      |

### 13. Policy owner (relating to these procedures)

#### Director of Nursing and Midwifery

Enquiries relating to this procedure may be directed to:

Title: Nurse Coordinator Palliative Care and Sub Acute

Department: Hospital Wide

Email: [Jo.Harris@health.wa.gov.au](mailto:Jo.Harris@health.wa.gov.au)

### 14. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

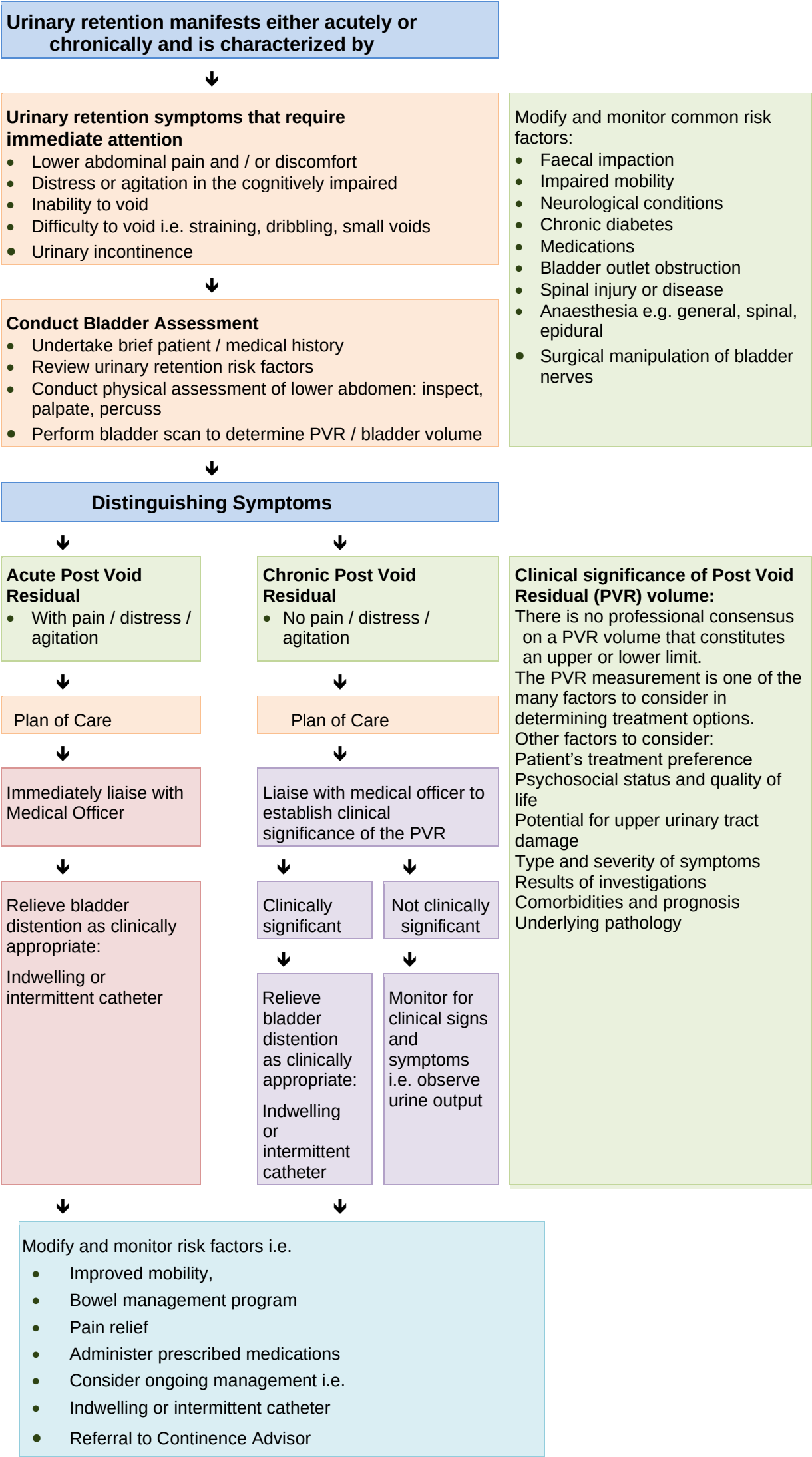
| Version | Effective from | Effective to | Amendment(s)                                                |
|---------|----------------|--------------|-------------------------------------------------------------|
| V3.1    | 17 02 2026     | 06/06/2027   | Minor update to update bladder scanning guide – section 4.2 |

### 15. Authorisation

This procedure has been authorised and issued by:

|                    |                                                          |
|--------------------|----------------------------------------------------------|
| Authorised by      | Russell Colyer-Cockburn - Director Nursing and Midwifery |
| Authorisation date | 17 02 2026                                               |
| Published date     | 17 02 2026                                               |

Appendix 1: Urinary Retention Management Flow Chart



Adapted from O'Connell (cited in SMHS Nursing Practice Standard: For the promotion of urinary continence, 2012).



## Appendix 2: Incontinence type by bladder assessment pattern

| Incontinence Type                                                            | Residual Volume                    | Leakage Occurrence                                                                                                                                         | Leakage Amounts                                         | Frequency                                      |
|------------------------------------------------------------------------------|------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------|------------------------------------------------|
| <b>Stress Incontinence</b>                                                   | Less than 100 mL                   | When patient coughs, sneezes, strains, or on standing.<br>Aware of need to void.<br>No urgency.                                                            | Generally small<br>Less than 50 mL                      | Usually normal<br>4-7 times per 24 hours       |
| <b>Urge Incontinence</b>                                                     | Amount varies                      | When patient coughs, sneezes, strains, or on standing.<br>Urgency with little or no warning of need to void.<br>Often unable to “hold on” without leakage. | Small to large                                          | Abnormal<br>Greater than 7 times per 24 hours  |
| <b>Overflow Incontinence<br/>Outflow obstruction<br/>Underactive bladder</b> | Often large<br>Greater than 200 mL | When patient coughs, sneezes, strains, or on standing.<br>Raised awareness of the need to void.<br>Urinary leakage overnight is a significant factor.      | Generally small<br>Less than 50 mL                      | Abnormal<br>Greater than 12 times per 24 hours |
|                                                                              | Large<br>500 – 2000 mL             | May or may not have leakage.<br>Urinary leakage overnight may be present.<br>No awareness of need to void.                                                 | Variable<br>May not leak urine at all                   | Abnormal<br>Will not experience need to void   |
| <b>Functional</b>                                                            | Amount varies                      | May or may not have leakage.<br>May or may not be aware.<br>Urinary leakage overnight may be present.                                                      | Small to large<br>depending on ability to access toilet | Unpredictable<br>May void inappropriately      |

- Many patients experience multiple problems relating to incontinence.
- Seek further guidance from Continence Advisor for issues relating to identifying Incontinence Type and patient management.

### Appendix 3: Appropriate interventions by incontinence type

| Incontinence Type                                    | Aggravating Considerations                                                                                                            | Treatment Options                                                                                                                                                             | Nursing Interventions                                                                                                                                  |
|------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Stress Incontinence</b>                           | Chronic cough<br>Obesity<br>Medications, Alpha blockers e.g. prazosin                                                                 | <a href="#">Pelvic floor muscle exercises</a><br>Oestrogen supplements: atrophic vaginitis<br>Surgery                                                                         | Liaise with medical staff, Continence Advisor or Specialist Physiotherapist.<br>Provide appropriate absorbent pads until management plan is effective  |
| <b>Urge Incontinence</b>                             | Urinary tract infection (UTI)<br>High caffeine intake<br>Low fluid intake for fear of incontinence<br>Constipation / faecal impaction | Treatment of aggravating considerations: UTI, appropriate fluid intake, bowel management<br><a href="#">Bladder training techniques</a><br>Drug therapy with anticholinergics | Liaise with medical staff, Continence Advisor or Specialist Physiotherapist.<br>Provide appropriate absorbent pads until management plan is effective. |
| <b>Overflow Incontinence<br/>Outflow obstruction</b> | Faecal impaction<br>Enlarged prostate<br>Urethral stricture<br>Pelvic organ prolapse                                                  | Treatment of aggravating considerations: UTI, Bowel management<br>Surgery<br>Permanent IDC<br>Alpha blockers                                                                  | Empty patients' bladder and manage <a href="#">Urinary retention</a><br>Liaise with medical staff, Continence Advisor or Specialist Physiotherapist.   |
| <b>Overflow Incontinence<br/>Underactive bladder</b> | Diabetes<br>Pelvic injuries<br>Neurological disorders: e.g. paraplegia, multiples sclerosis, diabetes mellitus                        | Monitor urinary output and post void residual volume<br>Indwelling Catheterisation or Intermittent / Self-catheterisation                                                     | Empty patient's bladder and manage <a href="#">Urinary retention</a><br>Liaise with medical staff, Continence Advisor or Specialist Physiotherapist.   |
| <b>Functional</b>                                    | Cognitive function disorders<br>Reduced mobility dexterity<br>Medications: Sedatives, loop diuretics<br>Environmental factors         | Improve environment<br>Medication review<br><a href="#">Bladder training techniques</a>                                                                                       | Liaise with medical staff, Continence Advisor or Specialist Physiotherapist.<br>Provide appropriate absorbent pads until management plan is effective. |

## Appendix 4: Bladder training techniques

- Bladder training is a non-invasive behavioral strategy used to manage symptoms of urgency, frequency and mixed incontinence.
- Bladder training, as described within this appendix may also be referred to as scheduled voiding regimens and are often used in conjunction with pelvic floor exercises.

| Technique               | Description                                                                                                                                                                                                                                                                                                                                                                                                                                     |
|-------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Bladder Calming</b>  | <p>Utilise relaxation or distraction techniques to postpone urination.</p> <p>Aims:</p> <ul style="list-style-type: none"> <li>• To improve control over urgency,</li> <li>• Increase bladder capacity,</li> <li>• Reduce incontinent episodes and improve patient confidence.</li> </ul> <p>Requires patient perseverance over several weeks.</p>                                                                                              |
| <b>Prompted Voiding</b> | <p>Actively reminding the person to initiate voiding at scheduled intervals.</p> <p>Aims:</p> <ul style="list-style-type: none"> <li>• To reduce the number of incontinent episodes and increase the number of patient-initiated toileting episodes.</li> </ul> <p>Requires significant caregiver motivation and commitment.</p> <p>Suitable for patients with cognitive and physical impairment.</p>                                           |
| <b>Habit Retraining</b> | <p>A toileting schedule matched to the person's usual voiding pattern.</p> <p>Aims:</p> <ul style="list-style-type: none"> <li>• To pre-emptively reduce the number of incontinent episodes.</li> </ul> <p>Requires significant caregiver motivation and commitment.</p> <p>Suitable for patients with cognitive and physical impairment, who have a consistent pattern.</p>                                                                    |
| <b>Timed Voiding</b>    | <p>A fixed voiding schedule set at regular times.</p> <p>Aims:</p> <ul style="list-style-type: none"> <li>• To avoid incontinent episodes through regular toileting rather than improve bladder function.</li> </ul> <p>Use temporarily for patients with impaired bladder filling / fullness sensations to avoid over distention e.g. post-epidural, post-natal, post-operative.</p> <p>Suitable for patients unable to toilet themselves.</p> |

### Undertake Continence Assessment

- All patients indicated for treatment with bladder training techniques require completion of the Three Day Bladder Diary.
- Analyse the continence assessment to determine usual voiding patterns.
- Once usual voiding patterns are identified apply the appropriate Bladder training technique.

# Implement Bladder Training

| Technique               | Instructions                                                                                                                                                                                                                                                                                                                                                                                                                         |
|-------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Bladder Calming</b>  | <p>Instruct patient each time an urge to pass urine is felt to:</p> <ul style="list-style-type: none"> <li>• Use a bladder calming technique.</li> <li>• Wait until urge is reduced.</li> <li>• Go immediately to the toilet.</li> </ul> <p>Advise patient to:</p> <ul style="list-style-type: none"> <li>• Walk calmly to the toilet, rushing may worsen the bladder sensation.</li> <li>• Treat each urge the same way.</li> </ul> |
| <b>Prompted Voiding</b> | <p>Prompt the patient to void at intervals consistent with their normal pattern.</p> <p>Provide positive reinforcement for:</p> <ul style="list-style-type: none"> <li>• Continent and self-initiated toileting episodes.</li> <li>• Correct reporting of wet/dry incontinence pads.</li> </ul>                                                                                                                                      |
| <b>Habit Retraining</b> | <p>Toilet the patient at intervals consistent with pattern identified from the continence assessment, and to pre-empt incontinent episodes.</p>                                                                                                                                                                                                                                                                                      |
| <b>Timed Voiding</b>    | <p>Toilet the patient at scheduled intervals e.g. two hourly.</p> <p>If continence pattern emerges refer patient to the Habit Retraining technique.</p>                                                                                                                                                                                                                                                                              |

Bladder calming techniques for consideration may also include, but are not limited to:

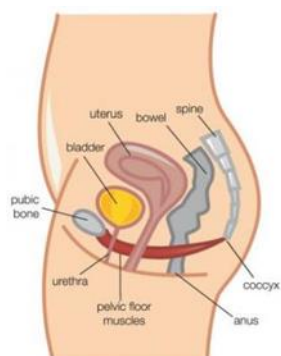
| Technique                                     | Instructions                                                                                                                                                                                                                                                                                                                                                                                                                   |
|-----------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b><u>Pelvic floor muscle contraction</u></b> | <ul style="list-style-type: none"> <li>• Hold pelvic floor muscle contraction until the voiding urge passes.</li> <li>• If unable to hold contraction long enough, attempt several quick lifts instead.</li> <li>• Use as each time an inappropriate voiding sensation is felt.</li> <li>• Use in conjunction with following techniques listed below.</li> </ul>                                                               |
| <b>Breathe Out</b>                            | <ul style="list-style-type: none"> <li>• Breathe out when urgency first occurs and continue to breathe normally.</li> <li>• Breathe holding and continued shallow breathing increases bladder pressure.</li> </ul>                                                                                                                                                                                                             |
| <b>Distraction</b>                            | <ul style="list-style-type: none"> <li>• Keep your mind busy with other thoughts and tasks.</li> <li>• The more urgency and the toilet is thought, the more urgency will increase.</li> </ul>                                                                                                                                                                                                                                  |
| <b>Direct pressure</b>                        | <ul style="list-style-type: none"> <li>• <b>Sitting:</b> on a firm surface e.g. arm of chair, edge of table or crossing legs while sitting in chair.</li> <li>• <b>Perineal pressure:</b> applying direct pressure to area between vagina and anus, if privacy permits</li> <li>• <b>Clitoral pressure:</b> place a firm finger/ hand pressure over the clitoris until voiding sensation ceases, if privacy permits</li> </ul> |
| <b>Toe curling</b>                            | <ul style="list-style-type: none"> <li>• Curl toes downwards as if making a fist. Either one or both feet.</li> <li>• Hold position until the urge has decreased.</li> <li>• Also use distracting thoughts e.g. poem, song, counting backwards.</li> <li>• Use with caution if patient experiences foot muscle cramps.</li> </ul>                                                                                              |
| <b>Calf stretches</b>                         | <ul style="list-style-type: none"> <li>• Keeping the knee straight, pull the foot up towards body.</li> <li>• Hold until the urge has decreased.</li> <li>• This can be undertaken sitting, standing or lying.</li> </ul>                                                                                                                                                                                                      |

|                            |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |
|----------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Sacral tapping</b>      | <ul style="list-style-type: none"> <li>• Tap, rub or press over the sacrum at the base of spine.</li> <li>• Keep tapping, rubbing or pressing until the voiding urge has passed.</li> </ul>                                                                                                                                                                                                                                                                                         |
| <b>Abdominal hollowing</b> | <ul style="list-style-type: none"> <li>• Draw in lower abdomen from below the umbilicus.</li> <li>• Combine with pelvic floor muscle tightening and hold as long as the voiding urge remains.</li> <li>• Don't breathe hold.</li> </ul>                                                                                                                                                                                                                                             |
| <b>Acupressure points</b>  | <ul style="list-style-type: none"> <li>• <b>Top lip:</b> place one finger in the middle of the top lip at the join between the lip and middle of the nose. Keep a gentle pressure until urge passes.</li> <li>• <b>Ankle:</b> place three fingers over the ankle bone on the outside of the leg. Keep pressure until the urge reduces.</li> <li>• <b>Suprapubic:</b> place four fingers on the left and right side above the pubic bone and hold until the urge reduces.</li> </ul> |

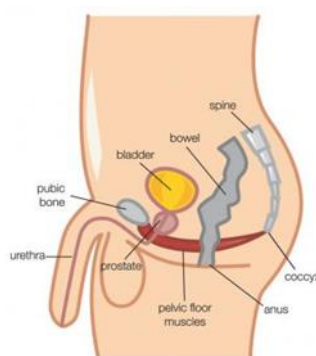
## Appendix 5: Pelvic floor muscle exercises

### Pelvic Floor Muscle Exercises Considerations

- Pelvic floor muscle exercises (PFME) are a proven non-invasive management technique that is recommended for women with stress incontinence, some men following prostate surgery and to assist with bladder training.
- Performance of the technique on a regular basis is designed to improve urethral resistance through strengthening of the pre-urethral muscles of the pelvic floor, primarily the pubo-coccygeus muscle.
- To enable patients to do pelvic floor muscle exercises effectively it is important that they are able to correctly identify the muscles that will need exercising.
- Patients who are unable to correctly identify their pelvic floor muscles, or do not respond to with the prescribed treatment; require continence assessment by the Specialist Physiotherapist.



Female pelvic floor muscles



Male pelvic floor muscles

\*Retrieved from: [Continence Foundation of Australia](http://www.continencefoundationofaustralia.org.au) 27/09/2013

- Patient information leaflets available from:
  - Site specific Continence Advisor or Specialist Physiotherapist.
  - Continence Foundation of Australia - Resources
  - Department of Health and Ageing website: [bladderbowel.gov.au](http://bladderbowel.gov.au) - Brochures